

What is Known About Reproductive Autonomy Among Justice-Involved Black Women? A Scoping Review

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Abstract

Objective: There are approximately 231,000 women detained daily within the nation's jail and prison systems with women of color making up nearly half of those experiencing incarceration. The purpose of this scoping review was to synthesize the literature on the reproductive autonomy of Black women influenced by incarceration, using the three tenets of reproductive justice. **Methods:** We searched PubMed, CINAHL, SocINDEX, and PsycINFO for research related to reproductive justice written in English and published in the United States from 1980 to 2022. A review of 440 article titles and abstracts yielded 32 articles for full-text review; nine articles met inclusion. **Results:** Eight addressed Tenet 1; five mentioned Tenet 2; none addressed Tenet 3. Recognition of the influence of incarceration on the reproductive autonomy of Black women is limited. **Conclusion:** The findings from this review suggest a need to address (a) reproductive choice, (b) support goals, and (c) support of justice-involved Black women.

Keywords

reproductive justice, discrimination, women's health, mass incarceration

Introduction

Mass incarceration, a phenomenon unique to the United States, began in the 1970s during the War on Drugs movement when three pieces of federal legislation mandated the steep financial sanctions and long blanketed sentences for low-level offenses (Heimer et al., 2023; Kajstura, 2019). These offenses have been predominantly associated with poverty and have disproportionately affected childbearing women (Heimer et al., 2023; Kajstura, 2019). Compared with other nations including authoritarian regions of the world, the United States comprises 30% of the world's incarcerated female population, twice the percentage of China and four times as much as Russia (Kajstura, 2018).

There are approximately 231,000 women detained daily within the nation's jail and prison systems and 763,425 on probation or parole (Heimer et al., 2023; Kajstura, 2019; The Sentencing Project, 2022). Table 1 lists the definitions of terms such as *jail systems*, *prison systems*, and *probation*. Historically designed for men, these settings do not provide for the gender-specific needs of women, such as family planning, pregnancy, childbirth, postpartum, and parenting-related health care (Hayes et al., 2020; Rothschild, 2019; Schlafer et al., 2019; Sufrin et al., 2015).

Much (47%) of this population are women of color (29% Black, 14% Hispanic, 2.5% American Indian and Alaskan Native, 0.9% Asian, and 0.4% Native Hawaiian and Pacific Islander) (Heimer et al., 2023; Kajstura, 2019; Statista, 2021) with Black women 1.7 times or 70% more likely to experience incarceration compared with non-Black women (Heimer et al., 2023). Thus, a significant subset of justice-involved Black women, meaning women who have experienced or are currently experiencing incarceration, may have limited ability to have their gender-specific needs met related to their reproductive autonomy (Hayes et al., 2020; Schlafer et al., 2019; Sufrin et al., 2015). Reproductive autonomy means the right for an individual to make choices about their body, sexual identity, and behaviors; their right to access reproductive and preventive health care; and their right to

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Table 1. Data Items.

Item	Definition
Reproductive justice	The human right to bodily autonomy, the right to have or not have children, and the right to parent the children one chooses to have in a safe and sustainable community (SisterSong, n.d.).
Reproductive rights	The composite of human rights addresses matters of sexual and reproductive health. Reproductive rights are protected by applying human rights in guidelines, national laws, constitutions, and regional and international treaties. While reproductive rights are instrumental to achieving population, health, and developmental goals, they are also important as human rights intended to protect the individual's inherent dignity. Reproductive rights consist of three broad categories of rights: (a) rights to reproductive self-determination; (b) rights to sexual and reproductive health services, information, and education; and (c) rights to equality and nondiscrimination (Erdman & Cook, 2008).
Reproductive autonomy	The power to control and decide about contraceptive use, pregnancy, and childbearing (Nguyen et al., 2019).
Parental rights	All of the legal rights and the corresponding legal obligations that go along with being the parent of a child, which include the right to legal and physical custody of the child; the right to physical access or visitation with the child; the right to inherit property from the child and to have the child inherit property from the parent; the right to consent to medical care and treatment for the child; the right to consent to the marriage of the child or its enlistment in military service; the ability to contract on behalf of the child; the obligation to provide financial support for the child; the responsibility to provide legal defense of the child in legal proceedings; the obligation to care for, direct, and supervise the child; the obligation to be legally liable for certain damages caused by the child; the obligation to see that the child attends school; and the obligation to protect the child and provide a safe living environment for the child (Law Insider, n.d.).
Bodily autonomy	The “fundamental human right” to contraceptive options, abortion, and childbearing (Judge et al., 2017).
Civil rights	These personal rights are guaranteed and protected by the U.S. Constitution and federal laws enacted by Congress (e.g., the Civil Rights Act of 1964). Civil rights include protection from unlawful discrimination and social and political freedom.
Human rights	The inherent rights that all human beings have regardless of gender, background, primary language, or any other characteristic. Inherent rights include access to life, safety, life-sustaining resources, and opportunities to live and thrive.
Childbearing individuals	Individuals who can conceive, be pregnant with, and give birth to children (Merriam-Webster, n.d.-a)
Women	Adult individuals are assigned female sex at birth (Planned Parenthood, n.d.)
Incarceration	The confinement in a jail or prison; the act of imprisoning someone or the state of being imprisoned (Merriam-Webster, n.d.-b)
Community supervision	An alternative to prison, with the two most common types being parole and probation. It is important to note that community supervision is implemented differently depending on the jurisdiction (Stand Together Trust, 2019).
Parole	The “conditional release of a prisoner” before the end of the prisoner’s sentence (Merriam-Webster, n.d.-c).
MAT	The combined use of medications, along with counseling and behavioral therapies, to treat opioid disuse (U.S. Food & Drug, 2019).
Jail	Holding facilities for women who are awaiting trial (Kajstura, 2019).
Prison	Prison systems reference those formally sentenced (Kajstura, 2019).

MAT = medication-assisted treatment.

decide when and how they have children (Hayes et al., 2020; Shlafer et al., 2019; Sufrin et al., 2015).

First defined in 1994, these concepts of reproductive autonomy reflect the intersectional framework of reproductive justice in a marriage of human rights and Black feminist theory (Ross, 2017). Guided by the following three tenets, reproductive justice gives voice to Black women: (a) the right to have children under the conditions of one’s choosing, such as bodily autonomy and self-agency over one’s body; (b) the right to have or not to have children, including one’s right to plan, prevent, or terminate a pregnancy; and (c) the right to parent children in safe, sustainable environments that are free from discrimination, violence, poverty, and fear for one’s safety

(Crawford et al., 2023; Hayes et al., 2020; Ross, 2017; p. 290; Ross et al., 2017; Shlafer et al., 2019). Thus, the high prevalence of Black women’s incarceration rates in the United States led us to conduct this scoping review to synthesize the extent and gaps within the literature on the reproductive autonomy of Black women influenced by incarceration in the United States, using the three tenets of reproductive justice.

Method

Scoping reviews offer a preliminary step in synthesizing the literature related to a specific phenomenon before conducting more rigorous investigations such as systematic reviews

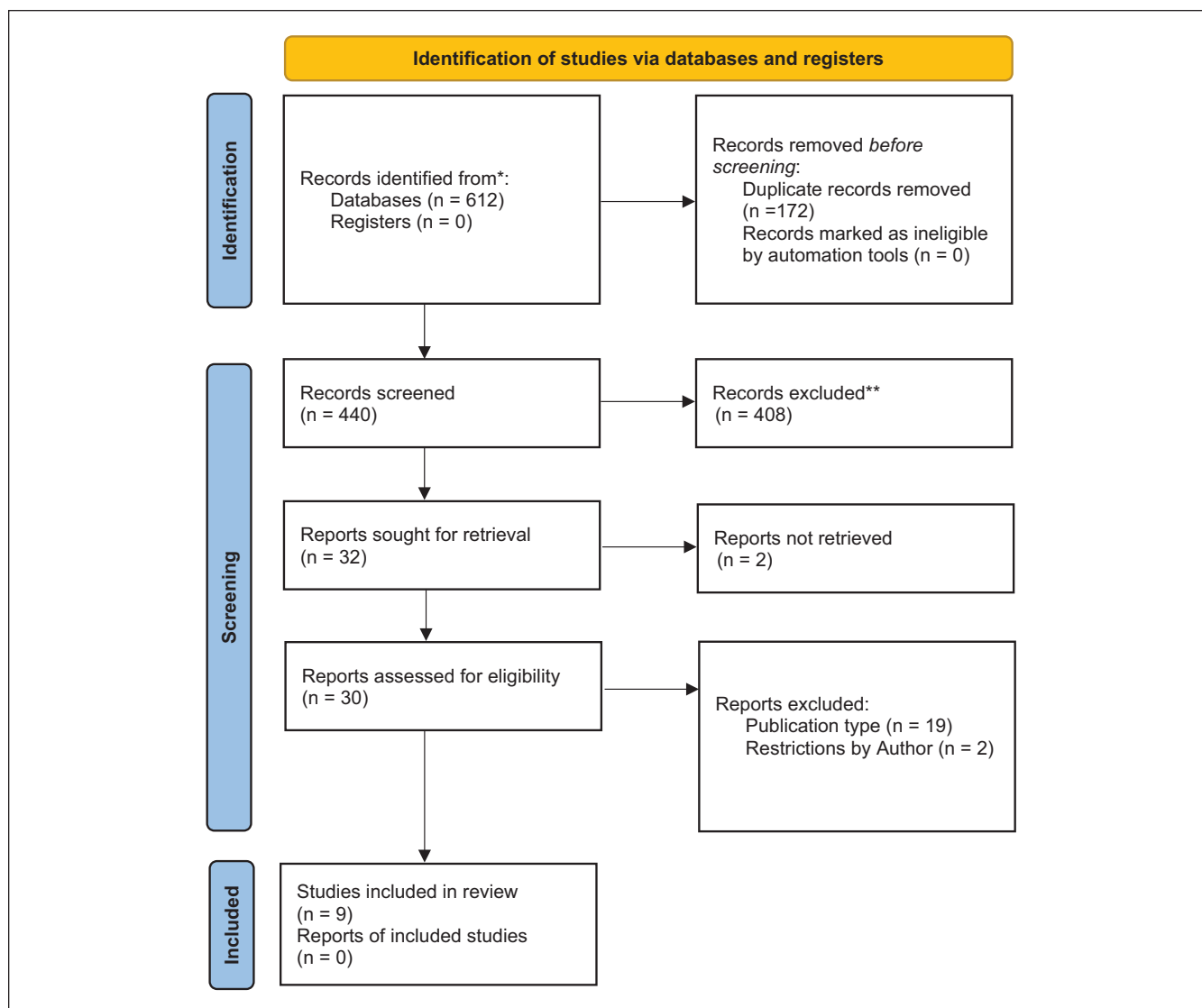


Figure 1. PRISMA Flow Diagram of Reviewed Studies.

Source. Adapted from Page et al. (2021).

(Tricco et al., 2018). In March 2022, the first author conducted a preliminary search within PubMed for any review articles that implicated the reproductive autonomy of Black women influenced by incarceration and found no review articles nor protocols that were registered. The Preferred Reporting Items for Systematic Reviews and Meta-Analyses for Scoping Reviews (PRISMA-ScR) (see Figure 1) was followed to report the findings (Tricco et al., 2018; Page et al., 2021) and to answer the research question, “What is the state of the literature and gaps related to the reproductive autonomy of Black women influenced by incarceration in the U.S. using the three tenets of reproductive justice?”

Search Strategy

For this scoping review, U.S.-based, peer-reviewed studies published in English between 1980 and 2022 were included.

This time frame was selected to match when the phenomenon of mass incarceration gained momentum within the United States (Kajstura, 2019). In the United States, incarceration and issues related to reproductive autonomy are unique compared with other regions of the world due to our policies and history of forced captivity of Black women (Kajstura, 2019; Ross et al., 2017). Data-based studies with qualitative and/or quantitative methods which focused on concepts related to the three tenets of reproductive justice in settings involving correctional oversight, such as jails, prisons, probation, parole, and medication-assisted therapy, were included. In the PubMed, CINAHL, SocINDEX, and PsycInfo databases, literature on the reproductive autonomy of women using the terms *pregnancy* OR *parenting*, OR *birth control*, OR *contraception* OR *reproductive justice* OR *reproductive rights* OR *reproductive autonomy* OR *parental rights* OR *bodily autonomy* OR *bodily autonomy* OR *civil*

rights OR human rights AND childbearing individuals OR women AND incarceration OR community supervision OR parole OR medication assistance therapy OR jail OR prison was used.

Our initial search yielded 612 articles, which we uploaded to Rayyan Qata Computing Research Institute (QCRI) (Ouzzani et al., 2016), a free online resource to assist literature synthesis. Removal of duplicates resulted in 440 articles. Two team members blindly reviewed the articles' titles and abstracts independently and selected those that met inclusion. A third team member resolved conflicts in selection. Reasons for articles not passing inclusion involved wrong publication types, such as commentaries, gray literature, books, and other nonresearch articles; location; or population ($n = 364$). In addition, 44 studies that focused on incarcerated female populations, including those ($n = 2$) related to prison nursery programs, were excluded because they needed to touch on concepts outlined by the three tenets of reproductive justice and were the wrong publication type. This process yielded 32 articles for full-text screening. Two reviewers read the full-text studies independently to determine inclusion. Conflicts ($n = 7$) were referred to a third reviewer for discussion, and consensus reached 13 studies for initial inclusion with 19 articles excluded for wrong publication type. Next, all four authors extracted data from the articles into tables using word. In this process, we did not retrieve four articles owing to restrictions from the author ($n = 1$), wrong publication type ($n = 2$), or wrong population ($n = 1$), thus resulting in $N = 9$ studies for final review.

Data Collection

Tables 2 to 4 provide a summary of the data extracted from the nine studies. These data consist of the studies' locations, carceral states (jail, prison, parole, probation), samples, the studies' aims, and methods (Table 2); the tenets of reproductive justice that the studies used, as well as their measures (Table 3); and qualitative themes and exemplars from Schonberg et al. (2020) and Wennerstrom et al. (2022) (Table 4). The team reviewed and confirmed the extraction process through discussion.

Results

Study Characteristics

This review synthesized the extent, range, and nature of the qualitative and quantitative data that are synthesized according to the three tenets of reproductive justice (Tables 2–4). Table 3 presents the studies in relation to the three tenets, with the studies' measures and recommendations. Eight of the nine studies addressed the right to bodily autonomy; five touched on the right to have or not have children; and no study explored issues related to the right to parent children in safe, sustainable environments. No articles that met criteria

controlled for sexuality or gender identity. All study samples comprised primarily women of color (see Table 2).

Tenet 1: The Right to Bodily Autonomy

Eight studies explored concepts related to women's right to bodily autonomy (Hemberg et al., 2021; Johnston et al., 2016; Pan et al., 2021; Schonberg et al., 2020; Sufrin et al., 2009; Thomas et al., 2019; Thompson et al., 2021; Wennerstrom et al., 2022). In the work by Hemberg et al. (2021), in which abortion-related knowledge was studied, respondents in Kansas City (KS/MO) and Birmingham (AL) had lower odds of high abortion-related knowledge than did respondents in Oakland (CA) (odds ratio [OR] = 0.19, 95% confidence interval [CI] = [0.10, 0.38] and OR = 0.17, 95% CI = [0.11, 0.28], respectively). This association remained after controlling for race and ethnicity, insurance status, and community supervision in the past year. In the work by Johnston et al. (2016), 86% of females from a detention facility reported that they had experienced sexual intercourse, with a mean age of onset of 13.8 years; 36% reported a past pregnancy. Interview data yielded four themes: (a) Detained adolescent girls had distinct priorities, behaviors, and motivations that affected reproductive health decision-making; (b) powerful external voices influenced reproductive health choices; (c) participants had difficulty in seeking services while "on the run"; and (d) detention could be a place for intervention and change. "A key finding of this study is that detained girls frequently had personal motivations that conflicted with following provider recommendations" (p. 52).

Pan et al. (2021) conducted a study with 22 state prisons and six jails; only six prisons and two jails had written policies for contraception, although 11 prisons and five jails allowed a choice to obtain permanent contraception such as sterilization. All facilities permitted postpartum procedures that included permanent contraception. Ten prisons and six jails reported providing access to reversible short-acting hormonal or long-acting reversible contraception (LARC). However, of these, only five prisons and five jails also provided access to permanent contraception. Conversely, six prisons allowed permanent contraception but did not give access to reversible contraception. Likewise, a variety of people, who included prenatal and hospital providers belonging to or unrelated to the prison or jail, obtained consent for permanent contraception.

Schonberg et al. (2020) addressed the needs and desires regarding contraception varied among the 10 interviewees influenced by incarceration. One was trying to conceive, others preferred condoms, and some had no desire to use contraception. However, all four participants who wanted contraception encountered systemic barriers to reproductive care (see Table 5). For example, one participant wanted an IUD and was waiting for approval from her insurance carrier. Similarly, many reported incarceration presented a

Table 2. Study Characteristics.

S. no.	Author/year	Location Carceral State	Sample	Study aims	Methods
1	Grella & Greenwell (2006)	California Parolees (aftercare program)	N = 483 women (aged 21–57) who are current substance users	To examine the relationships between variables correlated with the loss of parental rights, the ability to parent while incarcerated, and personal history of abuse and neglect. In addition, to gain insight into what individual characteristics impact the ability to parent following incarceration.	Quasi-experimental, two-group, pretest/posttest evaluation of the Female Offender Treatment and Employment Project
2	Henberg et al. (2021)	Oakland (CA), Kansas City (KS/MO), and Birmingham (AL) Oakland: Probation and community Kansas City: Recruited from city and county jail (no longer incarcerated at study time) Birmingham: Community correction facility	N = 381 women (aged 18–70) with criminal legal system involvement; 183 in Oakland, 58 in Kansas City, and 140 in Birmingham.	To examine how criminal system-involved women living in three areas of the United States demonstrate knowledge of abortion, including state laws and services.	Cross-sectional survey
3	Johnston et al. (2016)	Northern California Juvenile detention facility	n = 27 girls n = 28 participants completed the survey n = 27 participated in the interviews. Participants aged 14–19; 86% Latina. n = 22 n = 6	To explore barriers to the ability to access and implement reproductive health care services.	Anonymous survey and semi-structured interview
4	Pan et al. (2021)	AL, AZ, CO, GA, IA, IL, KS, LA, MA, MD, ME, MN, MS, OH, OK, PA, RI, TN, VT, WI Cook County (IL), Dallas County (TX), Hampden County (MA), Harris County (TX), New York City State prisons Jails		To describe permanent and reversible contraception policies at U.S. carceral institutions and the frequency of these procedures.	Cross-sectional survey and epidemiological surveillance study
5	Schonberg et al. (2020)	New York City Urban jail	N = 10 females Median age = 27 years Average time since incarceration, 21 months n = 4 Hispanic, 2 Black, 2 White, 1 multiracial, and 1 unknown; five had less than a high school diploma; five were unemployed; nine interviewees had been pregnant, with a median of two pregnancies; eight had children (M = 1); five had had at least one abortion.	To understand the contraceptive needs and pregnancy desires experienced by women after incarceration.	Qualitative, using semi-structured in-depth interviews

(continued)

Table 2. (continued)

S. no.	Author/year	Location Carceral State	Sample	Study aims	Methods
6	Sufrin et al. (2009)	United States Health care providers for Jail State prison Federal prison Juvenile center Other	n = 286 n = 63 n = 22 n = 1 n = 9 n = 5 The sample consisted mainly of White females, average age 45 years. Nurses comprised 57% of the respondents; 22% were doctors. Two-thirds were certified in correctional health, and two-thirds had been practicing at a correctional facility for 10 years or more.	To describe abortion access at correctional facilities. To identify potential barriers to incarcerated women obtaining pregnancy termination.	Survey
7	Thomas et al. (2019)	Women's Initiative Supporting Health Transitions Clinic, UR Medicine (University of Rochester) Local jail and women's prisons, supportive housing units, health fairs, SUD treatment programs, and in other community programs	N = 13 (11 had substance use disorder (SUD)) re-entering women recently released, attending a primary care clinic. Aged 26–61; six Caucasian, four African American, one Asian, one Alaska native or American Indian, one Native Hawaiian/Pacific Islander, one Hispanic	To describe women's experiences, including those with substance use histories, who participated in a pilot medical clinic for recently released women.	A qualitative exploratory study using semi-structured interviews
8	Thompson et al. (2021)	East coast Urban jail	N = 116 women, English-speaking, nonpregnant, aged 19–44 years (median age 29.5); 82 African American (70%), four White (21%), five Native American (4%), five Other Asian (4%), zero Pacific Islanders, and eight Latin X (7%). About 79% were Black, indigenous, or people of color. Majority with less than a high school education and pregnant at least once.	To explore incarcerated women's perspective of making provider-controlled methods of LARC available in a U.S. urban jail.	Mixed-methods, exploring contraceptive use and method choice before and after incarceration among women in a U.S. urban jail. Focus group discussions centered on incarcerated women's perceptions of LARC.
9	Wennerstrom et al. (2022)	Louisiana State prison	N = 22 women with a history of incarceration for at least 2 years or more in an LA state prison; 95% Black, mean age 50.7 years (range, 22–59). Mean age when incarcerated: 26.4 years (range, 13–41).	To describe formerly incarcerated women's experiences receiving health care during incarceration in Louisiana.	Qualitative

Note. LARC = long-acting reversible contraception.

Table 3. Tenet of Reproductive Justice and Measures.

S. no.	Author/year	Tenet of Reproductive Justice	Measures
1	Grella & Greenwell (2006)	Tenet 2: The right to have or not have children	Demographics and characteristics (e.g., employment status prior to incarceration), drug use and criminal behaviors, childhood adverse events and trauma captured using the Life Stressor Checklist-Revised (Wolfe et al., 1997), familial and/or social support measured using The Family Assistance Scale and The Social Support for Education/Training Scale, the number of children and parental status, Texas Christian University-Client Evaluation of Self at Intake to capture items such as social and psychological functioning, childrearing and parental attitudes using the Adult-Adolescent Parenting Inventory-2.
2	Hemberg et al. (2021)	Tenet 1: The right to bodily autonomy	Demographics and participant characteristics (e.g., health care system engagement and incarceration in the past year); abortion-related knowledge 10-item questionnaire that "chosen from previous research, in consultation with experts regarding abortion policy, and informed by summaries of legislation passed prior to May 2019 per information available via Guttmacher Institute" (p. 607).
3	Johnston et al. (2016)	Tenet 1: The right to bodily autonomy Tenet 2: The right to have children or not have children	Demographics, anonymous survey, and electronically recorded semi-structured interviews; surveys created with modified questions from existing instruments, including 10 questions from the California Health Initiative Survey (UCLA Center for Health Policy Research, 2012), 3 questions from the National Longitudinal Survey of Adolescent to Adult Health (Add Health, 2009), and 1 question from the Centers for Disease Control and Prevention National Health Interview Survey (CDC, 2012).
4	Pan et al. (2021)	Tenet 1: The right to bodily autonomy Tenet 2: The right to have children or not have children	Baseline survey of facility characteristics, the use of policies regarding female permanent contraception, who administered the consent for the use of permanent contraceptives and/or procedures, and who paid for them. The availability of LARC and the number of permanent contraception procedures performed more than 6 months were assessed.
5	Schonberg et al. (2020)	Tenet 1: The right to bodily autonomy Tenet 2: The right to have children or not have children	A designated reporter reported pregnancy outcomes for a 12-month time period. Semi-structured interview questions assessed contraceptive needs after the incarceration of people who identified as women, their pregnancy desires after incarceration, and their access to general and reproductive health care on re-entry.
6	Sufrin et al. (2009)	Tenet 1: The right to bodily autonomy Tenet 2: The right to have children or not have children	Survey with 26 multiple-choice and three open-ended questions on the degree of abortion access for those in correctional facilities. Questions addressed providers' attributes, abortion services provided or not provided, contraceptive counseling, and the provision and characteristics of general and reproductive health care for people identifying as women.
7	Thomas et al. (2019)	Tenet 1: The right to bodily autonomy	The WISH-TC uses the justice system and community stakeholders in its model, which has been shown to improve access to care and reduce stigma. Uses previously incarcerated community health workers who also previously received care at the clinic, trained in motivational and trauma-specific strategies to work with and support women entering the TNC—self-determination theory as a basis. Autonomy-supported communication acknowledges the client's perspective, encourages asking questions, offers treatment choices, and thus supports personal initiatives and competency. Autonomy support helps clients make healthy behavior choices. Competence supports self-efficacy in producing desired results. Relatedness: feeling respected and listened to in a caring manner. Semi-structured interviews about helpful/unhelpful experiences at the clinic with all personnel, strategies, and emotional reactions. Questions based on the sexually transmitted disease (STD) method and adapted from the Health Care Climate Questionnaire validated perceived autonomous motivation, which has been shown to mediate perceived competence, autonomous motivation, and healthy behaviors. Recruited before being released from incarceration and before re-entry.
8	Thompson et al. (2021)	Tenet 1: The right to bodily autonomy	Survey and focus groups. Five domains of the survey were conducted privately in the jail: demographics, health care access outside of jail, pregnancy history and attitudes, contraception history and perceptions of access in jail, and substance use history. Assessed pregnancy attitudes postincarceration with a validated scale. LARC questions assessed possible felt pressure by the jail to choose LARC, trust in jail health providers, and perceived ability for LARC removal. Also asked about the use and interest in non-LARC methods.
9	Wennerstrom et al. (2022)	Tenet 1: The right to bodily autonomy	2 focus groups of 11 women each who completed the survey. Conducted in a private room with no jail or health care personnel. Three domains: health care access, reproductive health needs, pregnancy, and contraception perceptions. Questions about access to LARC due to the potential for coercion and devaluing of women's reproduction. Before focus groups, an explanation of LARC and the need for access to providers. A 31-question semi-structured discussion guide (available upon request). Topics included experiences with accessing care, reproductive health needs, interactions with providers, medication use, and preventive care, among others.

Note. LRC = long-acting reversible contraception; TNC = transitions clinic network; WISH-TC = Women's Initiative Supporting Health Transitions Clinic.

Table 4. Qualitative Themes and Exemplars.

S. no.	Themes	Exemplar
Schonberg et al. (2020) related to Tenet 1		
1	Difficulty with obtaining an appointment	I really want [an IUD]. I don't want to get pregnant again. And I've been calling them and calling. I was supposed to have an appointment and I went to the hospital and they be like, "oh they changed your day." And I be like, "what you mean you change—you not even call me." (p. 196)
2	Providers' prevention as the barrier to receiving contraceptive care	[My OB/GYN] was going to try and take [my IUD] out but she was like it may be best for you to keep it in because it's been keeping you from getting pregnant. But I would like to take it out because I noticed that there is a lot of complications that I'm experiencing. So if she doesn't agree with that I have to go elsewhere. (p. 197)
3	Long wait times for Medicaid approval	Well, when I first came home I had to reapply for everything. I had to go to the Medicaid office, fill out all these forms, some of the forms weren't what they needed. I had to come back, and it was just like a mess. It took about two months, two and a half months. You know, you just got to wait. (p. 197)
4	Mistrust in health care providers	I don't really care [for my new doctor], because you don't care for people who don't care. Because people be there just for money because like you see there's a student for being a doctor so they pull them to see me to talk to me, and then she just be there to see me, say oh let me just sign this form for you, and that's it's just for money. (p. 197)
5	Barriers during re-entry related to accessing care	There should be a service provided before people are discharged and then even follow-up with the person. Find out what they have, what they don't have, what their needs are and if health insurance is one of them, "Let's get you some health insurance. Get that out the way and then we can help you with birth control." (p. 197)
Wennerstrom et al. (2022) related to Tenet 2		
1	Barriers to receiving appropriate care: Lack of access to care	"They don't send you to other opinions, to other doctors" (p. 10).
2	Barriers to receiving appropriate care: Lack of choice	The pap smear was horrible. I think it should only be women that do pap smears in a female institution. Only because, for one, I think it's more comfortable . . . So, you want me to come in here, in front this man, and just spread my legs, just have him just all up in there. No! Then, he was an older guy. He was rough . . . We have no choices in prison now. (p. 18).
3	Barriers to receiving appropriate care: Lack of preferences	I could tell him, "This medicine worked for me. Please put me on this." They'll have a problem with putting you on that. It would be the same thing that somebody else is getting there but that doctor will have a problem with me, saying "I can't get you the help like you did out there . . . This is prison. You're not a free woman." It was a battle. (p. 10).
4	Barriers to receiving appropriate care: Lack of concerns	. . . they get mad at you when you don't take it (medication) but they overdose me. I know my body with blood thinners, my cycle come down I just bleed, bleed, bleed . . . I'm on 13 days, and that's why I considered myself that my blood be too thin. And you tell them that and "its normal." That's not normal to me. (p. 13).
5	Barriers to receiving appropriate care: inadequate reproductive health care	"From 19 years, I got my pap smears. One time they started every 2 years because they said I didn't need it." (p. 14).
6	Barriers to receiving appropriate care: inadequate reproductive health care	"Well, I never had any Pap smears when I was incarcerated." (p. 17).
7	Health related conditions of confinement: lack of essential items	If the other women didn't get periods, they would just give them (pads) to us . . . Yeah, if you didn't have money, the supplies that they give you, it's not going to last a month . . . (It) is actually against the rules to find someone and like, "Hey, you have an extra bar of soap, can I get that?" No, you can't. You're not supposed to ask for that. (p. 18).
Schonberg et al. (2020) related to Tenet 2		
1	Pregnancy would interfere with their goals and priorities.	"I mean [pregnancy is] a blessing [but] I don't have stable housing. I'm unemployed. I'm technically uneducated. I'm single, so I don't have the foundation. I don't have the resources, the funding to take care of a child." (p. 195)
2	Pregnancy would interfere with their chances of successfully getting custody over their other children back.	In order for me to get custody of my son, I have to have an apartment so having a baby and not having your own place is going to be frowned upon by the judge, like you don't even have custody of your son, but you're having another baby. (p. 195).

barrier to contraceptive and reproductive health care because it disrupted established relationships with health care providers, insurance status, and contraceptive use in general. For example, most participants needed to apply or reapply for Medicaid postincarceration and experienced long approval times. Likewise, changing doctors and establishing a new relationship caused frustration and mistrust. Finally, participants felt that shortly before release, correctional facilities should provide assistance with insurance applications and finding community health care providers to facilitate access to reproductive care postincarceration.

Sufrin et al. (2009) surveyed correctional health providers to examine the provision of abortion: About 71% of respondents had counseled incarcerated people identifying as female at least once during their incarceration. The respondents were also more likely to work at facilities that allowed

inmates to seek elective abortions (84% provided counseling and dispensed contraceptives vs. 58% did not dispense or prescribe contraceptives). Furthermore, facilities that allowed people identifying as female to continue their preincarceration method of contraception were more likely to allow abortion than facilities that did not allow such continuation.

In Thomas et al.'s (2019) qualitative exploratory study, women receiving care at a transition clinic were mostly positive about their experiences. They stated that autonomy support from staff and being connected to staff while incarcerated helped empower them to look for health care and get their needs met quickly once released, as well as motivated them to set personal goals. Staff helped with scheduling appointments and with navigating the health care system. As a result, upon re-entry, women prioritized attending the clinic and

Table 5. Call to Action.**A Call for Reproductive Choice**

- 1 Acknowledge autonomy and agency regardless of incarceration status or history of justice system involvement.
- 2 Support shared decision-making and ensure benefits minimize harm by giving comprehensive education and including all contraception and reproductive health care options.
- 3 Address contextual barriers to reproductive health care and autonomy such as transportation; insurance coverage; continuity of care between jail, prison, and probation systems; forced or coerced sex work or other illegal activities for survival; food and housing insecurity; and the prevalence of violence and discrimination.

A Call to Support Reproductive Goals

- 1 Partner with justice-involved Black women by using a community-based participatory approach to one's practice by recognizing their right not only to prevent or stop pregnancy but also to be able to plan a pregnancy when they desire, regardless of incarceration status.
- 2 Establish trust by giving justice-involved Black women space to voice their concerns in a safe, nonthreatening environment.
- 3 Acknowledge and lessen power imbalances between the nurse, health care provider, or institution when interacting with justice-involved Black women.

A Call to Support Young Black Families

- 1 Allow flexibility related to one's reproductive autonomy and health care across several locations (jail, prison, and probation systems) by requesting medical records to be transferred promptly, creating a plan for their reproductive goals on admission to jail that follows them when they are moved to prison or released into the community.
- 2 Have timely data dissemination to inform legislation, research and program initiatives, and practice to identify the gaps better and address the health care needs for justice-involved Black women.
- 3 In research, control, and measure for concepts related to the three tenets of Reproductive Justice, especially Tenet 3: the right to parent in safe and sustainable environments.
- 4 In addition to addressing the needs of justice-involved Black women in jail and prison systems, they also address their unique needs during pregnancy, laboring, and parenting in the community while on probation that are congruent with their cultural norms and traditions.

viewed it as necessary, especially for substance use disorder recovery.

One woman in Thomas et al.'s (2019) study validated the support of autonomy: "They're really concerned about my health, my well-being, and me being successfully in recovery. And I like that, that gives me strength and courage to do what I need to do because I know I got people that's backing

me." The women also spoke of their feelings of competence in decision-making related to physical and mental health behaviors, even with relapse, and their feelings of relatedness by being respected, understood, and cared for nonjudgmentally by clinic personnel.

The findings of Thompson et al. (2021) for women in a U.S. urban jail suggested that 69% used public insurance and 79% had a regular health care provider before arrest. Finances, transportation, and child care were the main reasons for not seeking reproductive services. Within 30 days before their arrest, 28% were using nonbarrier methods, with 17% eligible for emergency contraception at the time of jail admission. Most of the women were pessimistic about becoming pregnant within the following year. Wanting LARC to be available in jail was expressed by 79%, with 26% interested in starting LARC before release. Reasons for wanting LARC were to prevent pregnancy (57%), the convenience of starting in jail (50%), and free cost (50%). Those who were not interested in LARC worried about side effects, distrusted jail staff, or wanted to become pregnant. Only 11% thought that they might experience coercion; 85% reported that they could get LARC removal in the community, with only 38% thinking that removal would be possible in jail. Regarding LARC, participants were mainly concerned about side effects or had misconceptions. Most women mistrusted jail health care provider competency and training. Facility cleanliness was an additional worry—most women preferred LARC placement from health care providers outside the jail. Loss of autonomy and coercion were not concerns, and the women viewed staff as nonresponsive to care needs.

In Wennerstrom et al.'s (2022) qualitative study of formerly incarcerated women, four main themes emerged: (a) issues related to barriers to receiving appropriate care, (b) quality and availability of health care services, (c) coping strategies and self-advocacy, and (d) health-related conditions of confinement (see Table 4). Among barriers to receiving appropriate care, women reported a lack of agency in the health care they received and said that their preferences or concerns received little consideration. Furthermore, the participants shared stories of inadequate reproductive health care. For health-related conditions of confinement, the findings supported the loss of bodily autonomy: The women reported having insufficient sanitation supplies while in the state prison and having to ration or go without basic hygiene-related items.

Tenet 2: The Right to Have Children or not Have Children

Five studies explored concepts related to the right to have or not have children (Grella & Greenwell, 2006; Johnston et al., 2016; Pan et al., 2021; Schonberg et al., 2020; Sufrin et al., 2009). In Grella and Greenwell's (2006) study of parolees, 33.1% of participants had lost parental

rights to one or more children, 66.9% had lost parental rights for all of their children, 18% had had a child removed from their care by child protective services, and a fourth of participants had given birth while incarcerated. Almost one-third (31%) of participants had children who had been adopted or were currently in the foster care system. More than half of the respondents (55%) had at least one child living with a grandparent. Concerning forms of contact (i.e., mail, in-person visits, and phone calls), 81% of participants reported having monthly contact with their children, 19% had less than monthly contact, and fewer than 1% indicated that they had had no contact with any of their children since being incarcerated.

Grella and Greenwell (2006) compared several characteristics between mothers in the loss and retention of parental rights. A higher proportion of mothers who had lost parental rights, as opposed to mothers who retained their rights, had not completed high school (58.8% vs. 46.4%); had never been legally employed (70.6% vs. 49.9%), with similar differences for those who had no vocational training (51.3% vs. 43.2%); had never been married (65.0% vs. 50.5%); had been adopted or had been in foster care themselves (31.9% vs. 15.7%, $p < .0001$); and grew up with substantial financial issues. In addition, the loss of parental rights was associated with giving birth to more children, with the odds increasing by 62% for each additional child. "Unexpectedly, higher scores on childhood problems and risk-taking were each related to less risky attitudes on two different scales" (p. 107). Ethnicity and lower educational attainment were associated with high-risk parental attitudes (e.g., corporal punishment).

In the work by Pan et al. (2021), one prison and one jail declined to report the number of postpartum permanent contraceptive procedures performed during the 6-month reporting period. However, the remaining 10 prisons and four jails reported that three women from two prisons and four women from two jails underwent postpartum permanent contraceptive procedures. All three women incarcerated in prison were in facilities that did not offer LARC. Three of the four women in jail who underwent permanent contraceptive procedures were from one jail that offered both the permanent procedure and LARC. The other woman in jail was in a facility that did not offer LARC and offered only permanent contraceptive procedures.

Schonberg et al. (2020) investigated the desire to become pregnant postincarceration (Table 4). Although most of those interviewed felt positive about pregnancy, many did not wish to become pregnant at the time of their interviews. Most felt that being pregnant would interfere with their goals and priorities, which included finding stable employment and housing and having a secure income. Two participants were delaying pregnancy because they anticipated the possibility of returning to jail due to open legal cases. Two others were seeking to regain custody of their children and felt that becoming pregnant would interfere with their chances of success.

In the work by Sufrin et al. (2009), 68% of the 286 respondents indicated that the inmates at their facility were allowed to obtain elective abortions if requested. Of these, 54% replied that the facility arranged appointments for the inmates. In contrast, the remainder reported that health care providers did not assist with making appointments. Several respondents indicated that the inmates held financial responsibility for the abortion, whereas others reported that their facility took care of every aspect of the process, including transportation and funding. According to 88% of the respondents, the facility arranged transportation to the abortion appointment, and 44% reported that inmates who requested an abortion received a referral to an options counselor.

Sufrin et al.'s (2009) findings implicated the accessibility of abortions for incarcerated people in relation to the dominant political party of the U.S. state of the correctional facility's location. Abortion services appeared to be more accessible in states with a democratic- or bipartisan-controlled legislature than in states with Republican-controlled legislatures. Only 50% of respondents with Republican-led legislatures indicated that abortions were allowed by their correctional facilities, whereas 78% of those with democratic-controlled legislatures and 72% of those with bipartisan-controlled legislatures said that their facilities allowed abortions.

Tenet 3: The Right to Parent in Safe and Sustainable Environments

No studies touched on this tenet of reproductive justice.

Discussion

Congruent to national statistics, the findings from the reviewed studies display disproportionate arrests (Heimer et al., 2023; Kajstura, 2019) and possible compromise of the reproductive autonomy of justice-involved Black women.

Tenet 1: A Call for Reproductive Choice

The reviewed studies revealed the limitation and gaps in research related to giving a reproductive choice among justice-involved Black women. This review mirrors prior literature on limited reproductive autonomy of justice-involved women (Crawford et al., 2022a; Hayes et al., 2020; Sufrin et al., 2015) and, most recently, the Supreme Court's decision over eliminating abortion health care in certain regions of the United States (Dickman et al., 2021). Powerful barriers associated with incarceration sometimes deter or prevent women from accessing emergency contraception, abortion services, or preventive health care of their choice, which can impede their reproductive autonomy (Crawford et al., 2022a; Karlsson & Zeilinski, 2020). Forcing women to continue with unplanned or unwanted pregnancies often result in these women having to engage in sex work or selling

illicit substances to financially support themselves and their babies, further inducing trauma, violence, and instances of rearrest, thus further limiting reproductive autonomy (Crawford et al., 2022a; Karlsson & Zeilinski, 2020).

Moreover, the findings from the reviewed studies reveal infrequent education and choice in contraception other than LARC. The most common types of contraceptives among women in the United States are female sterilization (18.6%) and oral birth control pills (12.6%) as compared with LARCs (10.3%) (CDC, 2019). However, evidence has shown that justice-involved Black women are encouraged to use LARCs as a first-line recommendation instead of to other forms of contraception (Brandi & Fuentes, 2020; Holt et al., 2020; Winters & McLaughlin, 2020). In a 2019 review of policies related to contraception in jail and prison systems, only 65% of systems had written policies related to contraception with limited authorization in giving choice/options to justice-involved women related to their birth control (Cheedalla & Sufrin, 2021) which further supports our findings that reproductive autonomy is limited in this population.

LARC placement has been the first line of family planning method given to high-risk or hard-to-reach groups for its high efficacy; however, despite LARC's many benefits, this method of birth control may not be the best for those who have experienced incarceration because of barriers to routine follow-up care, their state of health, or reproductive goals (Brandi & Fuentes, 2020; Holt et al., 2020). Contraindications specific to LARCs include pregnancy, uterine malformation, irregular bleeding, infection, and malignancies, and can enhance risks of irregular bleeding, ectopic pregnancy, pelvic pain, and perforation (Margaritis et al., 2023). Aside from LARCs, other forms of hormonal contraceptives also have adverse effects, such as weight gain, irregular bleeding, and mood changes which should be weighted and considered when counseling women on contraceptive use (Brandi & Fuentes, 2020; Holt et al., 2020).

Tenet 2: A Call to Support Reproductive Goals

Justice-involved Black women in the present review displayed a lack of shared decision-making in reproductive health care which may deter them from seeking care or trusting their health care providers. These data mirror other studies that describe the denial to justice-involved women of appropriate care that meets their reproductive goals by being insufficient, late, or stigmatizing (Crawford et al., 2023; Hayes et al., 2020; Sufrin et al., 2015). The denial to support Black women's reproductive goals reflects the historical underpinnings of slavery and reproductive control of Black populations in the United States, especially those influenced by incarceration (Brown et al., 2021; Ross, 2017). For example, in the past 40 years, women of color in the United States have been forcibly sterilized without informed consent (Marchevsky, 2018). Therefore, to overcome the ethical shortcomings of our past,

we must consider justice-involved Black women's experiences and goals related to childrearing in tandem and despite their prior or current incarceration status.

Tenet 3: A Call to Support Young Black Families

None of the studies included in this review touched on issues pertaining to the third tenet of reproductive justice. Failure by society to address Tenet 3 leads to a continuous cycle of adverse experiences for women and their children across the lifespan. As the number of incarcerated women has risen, so too has the need for prison, jail, and probation systems to address women's reproductive autonomy as it relates to family planning, pregnancy, childbirth, and postpartum care. Although prisons, jails, and probation departments do not systematically track pregnancy rates or birth outcomes, a 2019 national study found that 1 in 25 women entering prison was pregnant (Sufrin et al., 2017, 2019). Following birth, most incarcerated Black women experience the abrupt removal and separation from their biological children (Kwarteng-Amaning et al., 2019; Schlafer et al., 2019). Recently, there have been attempts to keep incarcerated mothers and their children together, with 12 states having policies that offer alternatives to incarceration that include prison nursery programs, pretrial diversion, and community-based reintegration programs (Kwarteng-Amaning et al., 2019; Pendleton et al., 2022; Schlafer et al., 2019).

Currently, as evidenced by our findings, there is a limitation on examining reproductive autonomy as it relates to the third tenet of reproductive justice for women in jail or prison systems and those on probation. The two articles we found related to prison nursery programs did not meet inclusion because they were commentaries or outside the United States and did not explore nor measure concepts related to Tenet 3 of reproductive justice. The limitation in research efforts to address concepts in Tenet 3 further reflects current conditions within our society that lack to provide a social and economic safety net for young families, such as the rise of gun violence in our schools and neighborhoods (Kolbe, 2020), as well as the recent shortage of infant formula, which has sent young families into panic as they try to nourish and protect their children (CDC, 2022). Research suggests that the economic stability and safety of communities result in the well-being of their children and future generations (CDC, 2021). Thus, implementing family-mother- and child-centered care models that include child care services, cost-effective or efficient delivery methods such as mobile health applications or that conduct simultaneous parent-child dyad care can give parents the ability to seek assistance while caring for their children (Crawford et al., 2023; Schlafer et al., 2019). These trauma-informed initiatives ideally should begin on admission to a correctional facility, extend throughout a woman's incarceration, and follow her after release while on community supervision.

Limitations

Scoping reviews have inevitable limitations, including mistakenly excluding relevant studies and not conducting a risk of bias assessment. However, using Rayyan, we have attempted to make this review thorough by conducting each stage of the review blindly, utilizing a tiebreaker, and discussing reasons for inclusion or exclusion by three subject matter experts. Our findings did not yield studies related to Tenet 3, which may be due to methodological error rather than a disregard from prior research; therefore, Tenet 3 warrants more investigation.

Recommendations for Culturally Congruent Care

Nurse has a pivotal role in mitigating the devastating effects of mass incarceration of Black women and the restrictions it has on their bodily autonomy. Nurses must take steps to offer culturally congruent health care to Black women involved with the justice system. For example, nurses must familiarize themselves of the power imbalances within systems and interpersonal interactions with Black women involved with the justice system and assist to recenter their needs and experiences as it pertains to their reproductive autonomy (McGlothen-Bell et al., 2023). Trauma-informed care such as understanding Black women's adverse experiences that span intergenerationally and across their lifespan leading up the justice involvement should be considered when addressing their needs and care (McGlothen-Bell et al., 2023). Promoting familial bonds, adequate pain management, breastfeeding support, and choices surrounding their reproductive health such as providing options around all forms of birth control should be provided regardless of incarceration status (Shlafer et al., 2019) for all Black women. Caution must be used when interacting with Black women with histories of incarceration such as avoiding stigmatizing words such as "addict," "convict," and "criminal" to prevent re-traumatization and stigmatization (McGlothen-Bell et al., 2023).

Shared decision-making about the reproductive health of Black women and their children should be implemented. Furthermore, nurses should foster a safe environment where authentic discussions about Black women's needs and goals can be communicated without fear of retribution (McGlothen-Bell et al., 2023). This means, giving resources to access preventive or acute health care services such as abortion care is imperative, especially, during this time of restrictive laws surrounding abortions across the United States. Barriers such as transportation, costs, and assistance with scheduling should be addressed and provided to prevent added health disparities surrounding abortion care.

Maintaining familial bonds should be a priority within the health care setting; however, when family separation is necessary, nurses should facilitate family communication. For example, nurses should create opportunities to exchange clothing items, photographs, human milk, and/or videos

(Shlafer et al., 2019) between Black women and their children involved with the justice system. Finally, visiting hours should be flexible with an understanding that the placement of children may be far and/or dependent on their next-of-kin care providers while families are separated due to maternal incarceration (Shlafer et al., 2019).

Conclusion

The findings from this review suggest a need to address (a) reproductive choice, (b) support goals, and (c) support of justice-involved Black women. More research is needed into how to measure this phenomenon specific to Tenet 3 of reproductive justice in justice-involved Black female populations to enrich data. Nurses are well poised to aid in advancing the reproductive autonomy of justice-involved Black women (McGlothen et al., 2023). Information provided by this review and in our call-out box (see Table 5) can help to inform recommendations for the provision of inclusive, humane, effective support, and culturally congruent health care by health care professionals and nurses when caring for justice-involved Black women and their young families.

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